



Cyclospora: When Should We Test or Treat?

Clinical Case Quiz

Note: These learning points apply to both pediatric and adult patients.

A previously healthy 30-year-old mother calls GCN for both herself and her 8-year-old child. Both have had four days of watery, non-bloody diarrhea, mild abdominal cramping, and fatigue. Neither has fever, vomiting, severe abdominal pain, or blood in the stool. Both are drinking well and urinating normally.

After hearing about the recent nationwide *Cyclospora* outbreak, the mother asks, "Should we both be tested, and do we need antibiotics?"

What is the next best step?

- Order Cyclospora testing immediately for both patients, because they have similar symptoms.
- Prescribe trimethoprim-sulfamethoxazole empirically to both patients.
- Manage as routine acute diarrhea with hydration, exposure assessment, and return precautions. Consider Cyclospora testing if the diarrhea becomes prolonged or relapsing.
- Direct both patients to an in-person appointment for examination and testing.



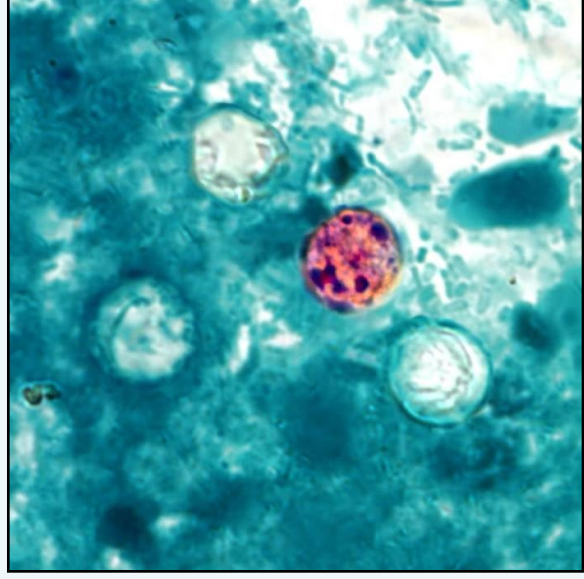
The answer is c.

Discussion: Choosing the Best Path for Care

The recent outbreak should increase our awareness of cyclosporiasis, but it does **not** mean that every patient with acute diarrhea requires stool testing, antibiotics, or in-person care.

Both patients have short-duration, uncomplicated diarrhea without dehydration or other red flags. They can generally be managed virtually with supportive care and close follow-up. The fact that two household members have similar symptoms may reflect a shared food exposure, but it could also represent routine viral gastroenteritis or another common cause of acute diarrhea.

Cyclospora Overview



Note. From *Cyclospora cayetanensis* [Photograph], by Centers for Disease Control and Prevention, n.d., CDC (<https://www.cdc.gov/php/Details.aspx?pid=7827>).

headache, low-grade fever, and other flu-like symptoms.

Incubation period averages one week (ranging 2 days to 2 weeks or more).

Background

A recent multistate cyclosporiasis outbreak has prompted increased patient concern locally. Although the identified illnesses have largely occurred outside California, clinicians should be prepared to recognize compatible symptoms, assess relevant food exposures, and order appropriate testing when indicated.

What is Cyclospora?

Cyclospora cayetanensis is a microscopic parasite transmitted through ingestion of contaminated food or water.

Common symptoms include frequent watery diarrhea, abdominal cramping and bloating, loss of appetite, weight loss, nausea, and prolonged fatigue.

Less common symptoms include vomiting, body aches,

When Should We Consider Cyclospora Testing?

Consider cyclosporiasis when a patient has:

- Immunocompetent patients with persistent diarrhea (>14 days)
- Returning travelers from areas where outbreak is occurring with persistent diarrhea (>14 days)
- Consider for immunocompromised patients with diarrhea.
- Testing is usually unnecessary for mild diarrhea lasting only a few days in clinically stable patients who are maintaining hydration.
- Prolonged diarrhea has a broad differential, so consider other bacterial, parasitic, medication-related, and noninfectious causes as well.

Do We Need to Change Routine Diarrhea Care?

Usually, no. Continue routine virtual management when the patient is otherwise well without red flags.

The current outbreak should primarily prompt a more focused **history that can guide whether testing for cyclosporiasis is indicated.** Ask about:

- Duration and frequency of diarrhea
- Whether symptoms improved and then returned
- Restaurant and produce exposures within the previous two weeks, compared with the latest outbreak advisories.

KPHC Ordering

When testing is indicated, use:

CRYPTOSPORIDIUM, CYCLOSPORA, AND CYSTOISOSPORA SMEAR [KPHC87207U]

- Expected turnaround time: **2–4 days**
- Transfer stool into container so that the contents reach fill line indicated on the vial.

Routine ova and parasite testing may not adequately evaluate for Cyclospora. Use the designated KPHC order when this organism is specifically suspected.

Can Testing Be Managed Through Virtual Care?

Yes. Stable patients without red flags generally do not require an in-person physician examination simply to submit stool testing.



Treatment of Confirmed Cyclosporiasis

Most immunocompetent patients will eventually recover without antimicrobial treatment. However, untreated illness may persist for a month or longer and may relapse.

Trimethoprim-sulfamethoxazole is the treatment of choice.

Dosing Guidelines	
Adult Dosage	TMP 160 mg plus SMX 800 mg (one double-strength tablet), orally, twice a day, for 7 days. Persons with HIV may need longer courses of therapy.
Pediatric patients ≥2 months to 18 years	8 – 10 mg/kg TMP and 40 – 50 mg/kg SMX per day, orally, in 2 divided doses for 7 days.

NOTE: If patient has a sulfa allergy, please send a Dr. Advice to ID to discuss next steps.

Key Takeaways

- Continue routine management for most short-duration acute diarrhea.
- Think of Cyclospora when watery diarrhea is prolonged (>14 days), particularly during May through August.
- Consider earlier testing in immunocompromised patients with compatible diarrhea.
- When testing is indicated, use the KPHC order: CRYPTOSPORIDIUM, CYCLOSPORA, AND CYSTOISOSPORA SMEAR [KPHC87207U].
- TMP-SMX is the treatment of choice for confirmed cyclosporiasis.

Further Reading on Cyclosporiasis

The regional infectious disease department [has created an FAQ](#) on the latest information regarding cyclosporiasis.

Acknowledgements

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